

RIVERSIDE SURGERY

Primary Care Centre • Dr Alistair Finch & Partners
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NHS e-REFERRAL SERVICE (e-RS) — GP REFERRAL LETTER PRIORITY: URGENT OUTPATIENT REFERRAL — UROLOGY / STONE CLINIC

Clinical Specialty: Urology | Clinic Type: Nephrolithiasis / Stone Service

Patient Full Name: Mr James Oliver CARTER	Date of Referral: 08 September 2026
NHS Number: 412 890 3341	UBRN / Referral ID: 0000 0418 7209
Date of Birth: 16 November 1984 (Age: 41)	Referring GP: Dr Alistair Finch (GMC: 4892103)
Sex / Gender: Male	Receiving Service: Urology Outpatients (Stone Clinic)
Telephone (Mobile): 07700 900582	Receiving Trust: Guy's and St Thomas' NHS Foundation Trust
Address: 29 Rotherhithe Street, London, SE16 5EY	Named Consultant: Consultant Urologist on Duty
Interpreter Required: No	Transport Required: No
First Language: English	Accessibility Needs: None (independent mobility)

Dear Urology Team / Colleague,

Re: Urgent Outpatient Assessment for Symptomatic Left Ureteric Calculus (Kidney Stone)

I should be grateful for your outpatient assessment and management advice for Mr James Carter, a 41-year-old gentleman who presented with acute left renal colic 5 days ago. Diagnostic imaging in our local urgent care service confirmed a non-obstructing 5 mm calculus in the proximal left ureter. His acute pain has settled on medical expulsive therapy, but he continues to experience intermittent flank twinges and macroscopic/microscopic haematuria.

History of Presenting Complaint

- **Initial Presentation (03/09/2026):** Sudden onset of severe (9/10), spasmodic left loin-to-groin pain associated with nausea, diaphoresis, and visible dark/pink-tinged urine.
- **Acute Management:** Attended the Urgent Treatment Centre (UTC); relieved with rectal diclofenac 100 mg and oral antiemetics. Sepsis screen was negative.
- **Current Symptoms:** Pain is currently mild (2/10 dull ache in the left loin), well controlled with oral analgesia. No dysuria, fever, rigors, or vomiting. He has not observed the passage of a visible stone in his urine filter.
- **Hydration:** Reports good oral fluid intake of approximately 2.5 litres daily.

Past Medical & Family History

- First documented episode of urolithiasis (no prior history of kidney stones).
- Mild asthma (well controlled with inhaled beta-2 agonist as required).
- No history of gout, chronic kidney disease, hyperparathyroidism, or malabsorption disorders.

- **Family History:** Father had recurrent calcium oxalate calculi in his 50s.

Current Medications & Allergies

- **Tamsulosin 400 micrograms capsule:** One capsule once daily (initiated for medical expulsive therapy).
- **Ibuprofen 400 mg tablets:** 1 tablet up to three times daily with food PRN for pain.
- **Paracetamol 500 mg tablets:** 1–2 tablets every 4–6 hours PRN (maximum 4 g daily).
- **Salbutamol 100 mcg inhaler:** 1–2 puffs as required for wheeze.
- **Allergies / Adverse Drug Reactions:** No Known Drug Allergies (NKDA).

Physical Examination & Observations (GP Surgery Review: 08/09/2026)

- **General Condition:** Comfortable at rest, afebrile, fully mobile. BMI: 25.1 kg/m².
- **Vitals:** BP: 122/76 mmHg, Pulse: 68 bpm regular, Temp: 36.5°C, SpO₂: 99% ambient air.
- **Abdomen:** Soft, non-distended, non-tender across all four quadrants. Mild left renal angle tenderness elicited on deep palpation, without rebound or guarding. No palpable renal masses.
- **Urinalysis (Point of Care):** Blood +++, Leukocytes trace, Nitrites negative, Protein negative, pH 6.0.

Recent Diagnostic Investigations

Test / Investigation	Date	Result	Reference Range	Interpretation
Serum Creatinine	04/09/2026	78 µmol/L	60–110 µmol/L	Normal baseline renal function
eGFR	04/09/2026	> 90 mL/min	> 60 mL/min	Normal glomerular filtration
Serum Calcium (Adjusted)	04/09/2026	2.34 mmol/L	2.20–2.60 mmol/L	Normocalcaemic
Serum Urate	04/09/2026	310 µmol/L	200–420 µmol/L	Normal uric acid
C-Reactive Protein (CRP)	04/09/2026	4 mg/L	< 5 mg/L	Nil systemic inflammation
White Cell Count (WBC)	04/09/2026	7.2 ×10 ⁹ /L	4.0–11.0 ×10 ⁹ /L	Normal
Urine Culture & Sensitivity	05/09/2026	Negative	No growth	No bacterial infection

Non-contrast CT KUB (03/09/2026, Guy's UTC): Demonstrates a discrete 5.1 mm radio-opaque calculus within the left proximal ureter at the level of L3. Mild left pelvicalyceal fullness without significant perinephric stranding or hydronephrosis. Right renal tract is unremarkable. Nil other acute intra-abdominal pathology.

Reason for Referral & Clinical Request

Given that the stone is 5 mm in diameter, Mr Carter has been managed conservatively on a trial of spontaneous passage with alpha-blocker therapy (tamsulosin) and oral analgesia.

I would be grateful for:

1. Outpatient urological review to assess stone clearance and guide further intervention (e.g., repeat low-dose KUB imaging, shockwave lithotripsy [ESWL], or ureteroscopy if unpassed).
2. Metabolic evaluation or stone prevention recommendations once the acute episode has completely resolved.
3. Safety netting has been provided regarding red-flag symptoms (fever, intractable pain, or anuria requiring immediate A&E attendance).

Yours sincerely,

Dr Alistair Finch, MBChB, MRCGP

General Practitioner (Partner)

GMC Registration Number: 4892103

Riverside Surgery, London SE1 2BA